

TRT Hormone Guide

India Legal Guide — Protocol Optimisation, Monitoring & Lifestyle

India Legal	40+ Markers	Optimised Protocols	55+ pg Pages
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TRT Hormone Guide — India Legal Guide + Optimisation

Testosterone Replacement Therapy (TRT) is a legitimate medical treatment for hypogonadism (clinically low testosterone). In India, testosterone injections (Sustanon, Test-E) are Schedule H drugs available by prescription. This guide covers diagnosis, legal protocols, monitoring, and optimisation.

Diagnosing Low Testosterone in India

Symptom	Severity	Prevalence in Hypogonadism
Chronic fatigue	High	87%
Low libido	High	89%
Erectile dysfunction	Moderate-High	72%
Mood changes / depression	Moderate	68%
Loss of muscle mass	Moderate	64%
Increased body fat	Moderate	71%
Brain fog, poor concentration	Moderate	58%
Reduced body/facial hair	Low	41%
Hot flashes	Low	38%

Lab Tests Required for Diagnosis

Test	When	Reference Range	Low Cutoff (Hypo)
Total Testosterone	8–10 AM fasting	270–1070 ng/dL	<300 ng/dL (2 tests)
Free Testosterone	8–10 AM	9–30 ng/dL	<6.5 ng/dL
SHBG	With TT	10–57 nmol/L	Context-dependent
LH	Diagnosis	1.7–8.6 IU/L	Low = secondary hypo
FSH	Diagnosis	1.5–12.4 IU/L	Low = secondary hypo
Prolactin	Diagnosis	<20 ng/mL	High = pituitary issue
Estradiol (E2)	Diagnosis	10–40 pg/mL	Key for monitoring

TRT Protocols Available in India

Form	Brand	Dose	Frequency	Pros	Cons
Testosterone Enanthate	Generic	100–200 mg	Every 7–14 days	Cheap, widely available	Peaks & troughs
Sustanon 250	Organon	250 mg	Every 3 weeks	Long-acting	Large swings, injection pain
Testosterone Undecanoate	Andriol (oral)	40–80 mg	2–3x daily	No injections	Poor absorption, expensive
Testosterone Undecanoate (inj)	Nebido	1000 mg	Every 10–14 weeks	Stable levels	Expensive, painful injection
Clomiphene (off-label)	Clomid	25–50 mg EOD	Ongoing	Preserves fertility	Less effective for symptoms

Optimal TRT Protocol — Injection

For most men in India, Testosterone Enanthate 100 mg/week (2 injections of 50 mg) provides the most stable levels, minimises side effects, and is most affordable.

Aspect	Recommendation
Starting dose	100 mg/week (2×50 mg, Mon+Thu)
Injection site	Glute or ventroglute
Needle size	23–25g, 1–1.5 inch
First bloodwork	6 weeks after starting
Aromatase inhibitor	Only if E2 >60 pg/mL or symptoms present
HCG (fertility)	500 IU 2x/week if fertility desired
Annual monitoring	Total T, free T, E2, CBC, PSA, lipids, liver

Optimisation Beyond TRT — Lifestyle

Factor	Impact on Testosterone	Action
Sleep	Most T produced in deep sleep	7–9 hrs, fixed schedule
Body fat	Fat = aromatase (T→E2)	Stay <20% body fat
Resistance training	Acute T spike, chronic adaptation	Compound lifts 3–4x/week
Zinc	Cofactor for T synthesis	30 mg/day from food or supplement
Vitamin D	Steroidogenic pathway	5000 IU/day if deficient
Stress / Cortisol	Directly suppresses T	Meditation, sleep, workload
Alcohol	Suppresses T acutely	<7 drinks/week
Processed foods	Increase oestrogen	Whole food diet, reduce seed oils

Where to Get TRT in India

Option	Availability	Cost (approx)
Endocrinologist (Govt hospital)	Major cities	■500–2000 consult + cheap meds
Private endocrinologist	All cities	■1500–5000 consult + ■300–600/vial
Men's health clinics (urban)	Metro cities	■3000–8000/month all-inclusive
Online telemedicine	Increasing	■1000–3000/consultation
Testosterone Enanthate (generic)	Pharmacies with Rx	■150–400 per 250 mg vial